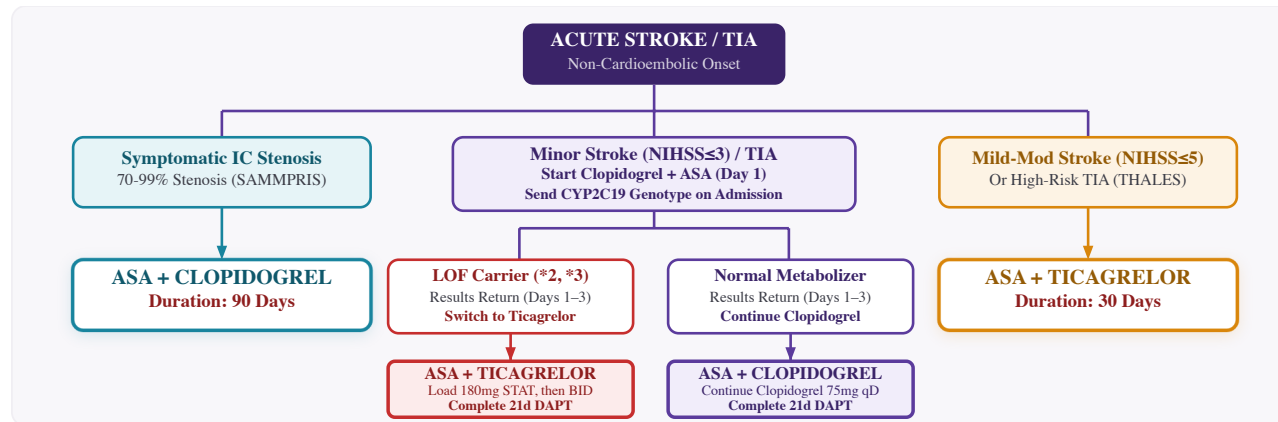


DAPT for Non-Cardioembolic Ischemic Stroke

Guideline-directed Dual Antiplatelet Therapy (DAPT) for secondary non-cardioembolic stroke prevention.



SCENARIO / TRIAL	TARGET POPULATION	LOADING DOSE (DAY 1)	DAPT DURATION	POST-DAPT
POINT Trial	NIHSS ≤3 or ABCD ² ≥4. **Within 12 hours** of onset.	Clopidogrel 600 mg + Aspirin 162–325 mg	Clopidogrel 75mg qD + Aspirin 81mg qD for 21 days	Aspirin 81mg
CHANCE / INSPIRES	CHANCE: NIHSS≤3, ABCD ² ≥4 within 24h. INSPIRES: NIHSS≤5, ABCD ² ≥4, ≥50% stenosis within 72h.	Clopidogrel 300 mg + Aspirin 75–300 mg	Clopidogrel 75mg qD + Aspirin 75–100mg for 21 days	Clopidogrel 75mg (to Day 90)
CHANCE-2 Trial	CYP2C19 LOF carrier (*2/*3) + Minor stroke/TIA. **Within 24h**.	Ticagrelor 180 mg + Aspirin 75–300 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 21 days	Ticagrelor 90mg BID (to Day 90)
THALES Trial	NIHSS ≤5 or high-risk TIA (ABCD ² ≥6 or symptomatic stenosis) within 24h.	Ticagrelor 180 mg + Aspirin 300–325 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 30 days	Aspirin 81mg
SAMMPRIS Trial	Severe symptomatic stenosis (70–99%) of major intracranial artery.	Aspirin 325 mg + Clopidogrel 75 mg (no load)	Clopidogrel 75mg qD + Aspirin for 90 days	Aspirin

CYP2C19 Genotyping & Clopidogrel Resistance

- ~30% of US stroke patients carry a loss-of-function (LOF) allele (*2, *3), resulting in poor active metabolite generation and a 2x stroke recurrence rate.

Just Safety

- **Bleeding vs. Benefit**: DAPT beyond 21 days for minor stroke/TIA increases bleeding (including ICH) without reducing recurrence (POINT/CHANCE).
- **Post-Lytic Policy**: Avoid DAPT in patients who received TNK or EVT (unless specifically requested by neurointerventions for stent).

POINT: Johnston SC et al. *N Engl J Med.* 2018;379:215–225. | **CHANCE:** Wang Y et al. *N Engl J Med.* 2013;369:11–19. | **CHANCE-2:** Wang Y et al. *N Engl J Med.* 2021;385:2520–2530. | **INSPIRES:** Gao Y et al. *N Engl J Med.* 2023;389:2413–2424. | **THALES:** Johnston SC et al. *N Engl J Med.* 2020;383:207–217. | **SAMMPRIS:** Chimowitz MI et al. *N Engl J Med.* 2011;365:993–1003.